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Name: دعاء خالد سيد محمد National ID: 29705061400861	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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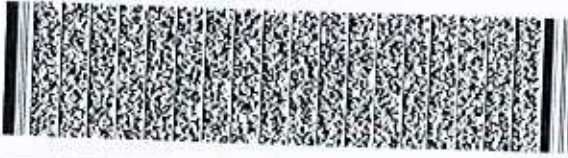
وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدى و تحتاج الى Contrast-enhanced CT of the brain.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

٢٠٢٢/١٠ ٢٩٧.٥.٦١٤.٠٠٨٦١
ربة منزل

أنثى
مملعة
متزوجة
اسلام محمد عبدالواحد محمود
البطاقة سارية حتى ٢٠٢٩/١٠/١٤ ١١



جمهورية مصر العربية
بطاقة تحقيق الشخصية

دعاء
خالد سيد محمد
الاحميين
مركز القناطر الخيرية - القليوبية

١٩٩٧/٠٥/١١ ٢٩٧.٥.٦١٤.٠٠٨٦١
JP4459237





Patient Name : الأستاذ / دعاء خالد سيد محمد	Request No. : 13008616
Patient No. : 29705061400861	Request Date : 10/03/2025
Path No. : 971/25	Verification Date : 20/03/2025
Sex : Female	Financial Category : أورام دار السلام - مبادره
Age : 27 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data

Known case of right breast cancer status post MRM showing Left breast mass

Nature of specimen

Tru cut biopsy, collected at 10/3/2025, 10:30 am

Gross examination

Multiple grayish white tissue cores up to 1.5 cm, totally submitted.

IHC technique: Sections prepared from paraffin block for immune-staining using Bench

Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination

- * Histological type: Invasive duct carcinoma
- * Grade (differentiation +nuclear pleomorphism+ mitosis): (3+2+2) Grade II
- * DCIS: No
- * Angiolinvasion: Couldn't be detected in the received biopsy
- * Perineural invasion: No
- * IPT (Positive control showed appropriate reactivity) :
- * ER: Moderate nuclear reaction in 25 % tumor cells (Allred score 5/8)
- * PR: Weak nuclear reaction in 20 % tumor cells (Allred score 4/8)
- * HER2: Strong complete membrane reaction in >10 % of tumor cells (score 3+)
- * Ki 67: 35 % nuclear reaction of tumor cells

Diagnosis

Left breast mass, Tru cut biopsy:

Verified by :

Dr.Heba Hasan



Name: دعاء خالد سيد MR. : 846	
Detailed Medical Report	
السيد	
تحتية طبية وبعد,,	
	إيماء للطلب المقدم من بشأن تقرير طبي للمريض
	بقسم: رقم طبي:
ننشر بأن نوضح فيما يلي تقريراً عن حالتها	
	تاريخ وساعة الدخول : تاريخ وساعة الخروج :
وبعد الكشف والفحوصات والمتابعة وجد انها The patient is a 29-year-old with triple-negative breast cancer diagnosed in 2023, treated with radiation to the right mammary region in 2025, and subsequently developed a local recurrence with bone metastases. The patient has received taxol and heceptin intravenously, and is currently on oral xeloda and zoledronic acid (Zometa) for three months starting February 2026. Multiple scattered osseous lesions are present in the right frontal bone, sternum, dorsal and lumbar vertebrae, with a CT-guided core biopsy confirming metastatic disease. The patient is now on a regimen of oral xeloda 500 mg twice daily for 14 days per cycle, combined with tykerb (5 tablet daily) and zoledronic acid 4 mg. A CT scan of the chest, abdomen, and pelvis was performed and is pending comparison. The patient reports lower back pain and bilateral femoral pain, and has been referred to a pain clinic for management. A recent CT of the chest, abdomen, and pelvis (02 June 2026) shows progressive osseous disease despite the current xeloda 500 mg 3 × 2 × 14 day schedule and supportive therapy. Laboratory results were accepted and are pending a new multidisciplinary team decision. The patient continues on the same xeloda 3 × 2 × 14 day regimen and tykerb 5 tablet daily until a new line of treatment is approved. In August 2026, the multidisciplinary team recommended a trial of herceptin IV plus navelbine until approval of Enhertu. An MRI of the lumbosacral spine was ordered, and palliative radiotherapy was planned. A bone scan in August 2026 still shows old osseous lesions at the previously involved sites, with new lesions noted in the proximal right femur and distal left femur. The patient is scheduled to receive herceptin IV and navelbine 90 mg for cycle 1. On 10 September 2026, the patient received herceptin IV and navelbine, and an abdominal ultrasound was performed, revealing AST 214 U/L and ALT 150 U/L, indicating hepatic support is required.	
تحريراً في ,	
	التاريخ : 21-09-2026
	الطبيب المعالج : نهى سعيد علي محمد
يعتمد ,	



Patient Name : الأستاذة / دعاء خالد سيد محمد
Patient No. : 297050-1400861
Path No. : 971/25
Sex : Female
Age : 27 Y
Referring Doc :

Request No. : 13008616
Request Date : 10/03/2025
Verification Date : 20/03/2025
Financial Category : اورام دار السلام - مبادره
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

Invasive duct carcinoma, Grade II

IPT:

- * ER: Positive (score 5/8).
- * PR: Positive (score 4/8).
- * HER2: Positive (score 3+).
- * Ki67: Positive in 35% of tumor cells.

Signature

Dr. Rana Fathy

Dr. Heba Hasan

Verified by :

Dr. Heba Hasan

Printing Date : 10/04/2025 11:49:40

Page 2 of 2

Printing By : Mrs. Mayada



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, E.

????, ??? Patient ID: 10110225010999 DOB: 06-May-1997 Date: 23-Jun-2025

Patient Lastname: خالد

Patient Firstname: دعاء

HIS Patient ID: 10110225010999

RIS Patient ID: 371272

Date of Birth: 06/05/1997

Procedure: 70030, Bone Scan

Medical Question:

Clinical Comment:

===== REPORT TEXT =====

99mTc MDP Bone Scan Report (23/6/2025)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of 99mTc MDP
Revealed:

" No definite evidence of active osseous deposits could be seen over the surveyed skeleton.

" Degenerative changes are noted over the dorso-lumbar vertebrae.

" Arthritic changes are noted over both shoulder and both knee joints.

" Both kidneys are visualized.

Conclusion:

" No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obligated

Areej Atran

Omnia talaat,MD

===== REPORT TEXT ENDE =====

Patient Name: Doaa Khaled Sayed Mohamed	Date : 20/07/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 28 Year(s)
Medical ID: 838/2023	Pat. ID: 29777

LEFT MAMMOGRAM AND BREAST ULTRASONOGRAPHY REVEALED:

Indication: Follow up of pathologically proved left breast carcinoma since 1 year just started chemotherapy and right mastectomy .

- **Breast density:** Moderate dense breast tissue (masking the underlying parenchyma) . *Breast density matches ACR d.*
- **Increases size of the left UIQ malignant mass .**
- Normal LEFT skin and nipple.

By complementary breast ultrasound:

- **Multiple irregular shaped masses are seen in between 10 o'clock position of the left breast to 2 o'clock position of the left breast ,the largest is seen at 12 o'clock position ,zone 3 measuring (20 x 20 mm) ..**
- **Clear right mastectomy bed.**
- **Multiple suspicious looking left axillary (levels 1 and 2) the largest measures (23 x 14 mm).**
- **About 3 suspicious looking right lymph nodes, the largest measures (10 x 6 mm).**

Conclusion:

- Comparing to the previous radiological datafindings are suggestive of progressive disease **(PD)** (BIRADS 6 category).

Much Obliged

DR.MONA ZAKY.MD

Associate professor-Breast imaging-Mansoura university

Signature Dr. Mona Zaki

20/07/2025 4:11 PM



Patient Name: Doaa Khaled Sayed Mohamed	Date : 20/03/2023
Exam : ultrasound BREAST (BOTH SIDE)	Age : 25 Year(s)
Medical ID: 838/2023	Pat. ID: 29777

Combined bilateral breast mammography and complementary ultrasound

Indication:

Right breast palpable lump.

Report:

- Extremely dense breast parenchyma (ACR D)
- Right breast UOQ and LOQ suspicious calcifications cover an area measures about 8 x 8.5 cm in MLO view.
- Complementary ultrasound revealed right breast multiple irregular spiculated hypoechoic mass lesions, , seen at 6 to 10 o'clock, largest measuring 2.6x5cm,
- Normal skin thickness, and nipple-areola complex of the left breast.
- Indeterminate level I and II axillary lymph nodes are seen with muffled hila, measuring 0.8 cm
- Nonspecific left axillary LN.

Opinion:

- Right breast multicentric malignant looking mass lesion (BIRADS 6)
- Indeterminate right axillary LNs.
- Benign findings left breast (BIRADS 2)

Yours truly

Signature Dr. Aya Deabes

20/03/2023 2:19 PM



Patient Name: Doaa Khaled Sayed Mohamed	Date : 30/08/2023
Exam : ultrasound BREAST (BOTH SIDE)	Age : 26 Year(s)
Medical ID: 838/2023	Pat. ID: 29777

Targeted ultrasound on right axilla

Indication:

- Patient with pathologically proven right breast carcinoma.

Report:

- Still noted one axillary lymph node at level I showing focal cortical thickening about 4.1mm and preserved fatty hilum , measuring about 20x8mm.
- Free supra and infra clavicular regions.

Signature Dr. Nehal Hamdy

30/08/2023 1:55 PM

Patient Name: Doaa Khaled Sayed Mohamed	Date : 28/10/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 28 Year(s)
Medical ID: 838/2023	Pat. ID: 29777

Non-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

As compared to 27/2/2025, The current revealed:

Chest:

- Right mastectomy showing no definite residual or recurrent masses.
- Diffuse left breast asymmetrical parenchymal heterogenous density .
- Few left axillary enlarged lymph nodes (level I and II) are seen, The largest measures about 2x1 cm.
- Few small and tiny mostly subpleural pulmonary nodules the largest right lung lobe fissural based about 3 mm.
- Multiple enlarged peritracheal and prevascular lymph nodes , the largest measures 2.2x1.5cm prevascular LN .
- ***Otherwise***, No sizable pulmonary masses, consolidations or cavitary lesions.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Multiple prominent para aortic and mesenteric lymph nodes are seen the largest left par aortic about 1.3 cm, on non contrast basis.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- IUCD is seen in place.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.

Patient Name: Doaa Khaled Sayed Mohamed	Date : 28/10/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 28 Year(s)
Medical ID: 838/2023	Pat. ID: 29777

- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

The scanned skeleton:

- Newly developed D2 , D8 and D11 and L1 lytic / sclerotic lesions .
- Redemonstration of L2 mixed lytic /sclerotic lesion . No extra-osseous soft tissue component
- L5 grade I lytic spondylolisthesis.

OPINION:

Status post right mastectomy showing :

- **Progressive course regarding bone metastasis .**
- **Stationary few small pulmonary nodules .**
- **No definite hepatic metastatic deposits on non contrast basis.**

Signature Reeham Khader
16/11/2025 11:18 PM

Name : Doaa Khaled Said Mohammed
Branch : Dokki
Date : 28-08-2026 07:26
Medical No. : 449316

Ref. Physician Prof : Goustav Roussey Egypt

Tc-99m MDP FOLLOW UP BONE SCAN

History

29 year-old female patient with history of metastatic right breast cancer to the bone, complains of bone ache and referred for follow up.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings

- *Considering the last bone scan report dated 9/2/2026, the current scan revealed still noted active osseous lesions which were located at the frontal bone of the skull, sternum, dorso-lumbar vertebrae, bilateral scattered ribs, pelvic bones and lesser trochanter of the left femur.*
- Newly developed lesions at the proximal part of the right femur and distal part of the left femur.

IMPRESSION

- Still noted old osseous lesions at the aforementioned sites with newly osseous lesions at the proximal part of the right femur and distal part of the left femur, for CT correlation.
- Follow up bone scan is advised to monitor the therapeutic response.

MUCH OBLIGED
Dr. Alaa Nagy, MD
Nuclear medicine


 جمهورية مصر العربية
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 دعاء
 خالد سيد محمد
 الاخمين
 مركز القناطر الخيرية - القليوبية
 ٢٩٧/١٠
 ٢٩٧.٥.٦١٤.٠٠٨٦١
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 ربة منزل
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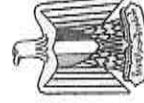
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ربة منزل



متروجة
مسلمة
اسلام محمد عبدالواحد محمود
البطاقة سارية حتى ٢٠٢٩/١٠/١٤

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Branch : Dokki
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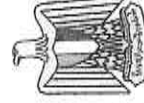
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متروجة
مسلمة
اسلام محمد عبدالواحد محمود
البطاقة سارية حتى ٢٠٢٩/١٠/١٤

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ECHOCARDIOGRAPHIC REPORT

Name	Ms. / Doaa Khaled
Date	24 September 2026
Ref. Dr.	Prof. /

Dear colleague ... Prof. Dr./

many thanks for referring your patient

M- mode measurements					
LVEDD	3.7	(3.5 - 5.6) cm	LEFT ATRIUM	2.2	(2.0 - 4.0) cm
LVEDS	2.2		AORTIC ROOT	2.4	(2.0 - 4.0) cm
LVSWT	0.7	(0.6 - 1.1) cm	AO. EXCURRATION	1.7	(1.5 - 2.5) cm
LVPWT	0.7	(0.6 - 1.1) cm	R. VENTICLE	0.8	(0.6 - 2.3) cm
EF	71 %	(> 55 %)	FS %	39 %	(25 - 40 %)

Color coded Doppler Study

- Mitral Valve : Normal flow , Mitral valve area = 4.3 cm² by pressure ½ time .
- Aortic valve : Normal flow .. Maximum pressure gradient = 6 mmHg
- Pulmonary valve : Normal flow, Maximum pressure gradient = 3 mmHg
- Tricuspid valve : Normal flow ...

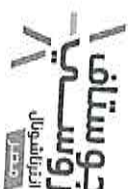
Comment :

- Normal all cardiac dimensions ..
- No segmental wall motion abnormality ,, at rest ..
- Good left ventricular systolic function .. **Ef. = 71 % .. at rest ..**
- Good L. V diastolic function ..
- Normal right side of the heart .
- Intact inter -atrial and inter –ventricular septa
- Normal all valvular flow and shapes .

Opinion : Echo-Doppler study of : Normal heart .. at rest.

Much obliged .Dr. Emad Sobhy
Consultant Cardiologist

Medical No. : 9168
Patient's Name: Doaa Khaled Said Mohammed
Date: 2026-09-22
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt



C.T. SCAN OF THE BRAIN WITHOUT I.V. CONTRAST INJECTION
REVEALED:

- Multiple bilateral cerebral and brainstem intra axial heterogeneous hypodense SOLs are seen largest measuring 31 x 32 mm at the right high frontal region. They show multiple hyperdense foci within. They are seen associated with surrounding edema and causing mass effect in the form of effacement of the adjacent cortical sulci and compression of the right lateral ventricle with mild midline shift to the left side.
- No extra-axial collections.

Conclusion:

- Multiple bilateral cerebral and brainstem intra axial SOLs likely metastatic considering patient's medical history.

Dr.sherif Wagdy

Name : Doaa Khaled Said Mohammed
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 28-08-2026 07:26
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Nuclear medicine

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بطاقة تحقيق الشخصية



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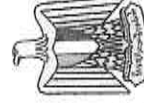
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متروجة
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